

## Hospital Discharge Summary

Patient: Dorothy Williams

Admission date: September 2, 2026    Discharge date: September 4, 2026

Service: Cardiology

Diagnosis: Atrial fibrillation with rapid ventricular response, resolved.

Hospital course: Ms. Williams, an 82-year-old with a history of atrial fibrillation, chronic kidney disease stage 3, type 2 diabetes, and osteoarthritis, was admitted with atrial fibrillation with rapid ventricular response. Rate control was achieved with adjustment of her home regimen. Warfarin was increased from 5 mg to 7.5 mg oral once daily given a subtherapeutic INR on her prior dose; INR to be rechecked in one week. Furosemide was held during the admission due to transient hypotension and restarted at discharge at a reduced dose of 20 mg oral once daily, down from her home dose of 40 mg. Per nephrology, ibuprofen was discontinued given its contribution to her declining renal function; acetaminophen 500 mg oral as needed was started in its place for osteoarthritis pain.

### Discharge medications:

- Warfarin 7.5 mg oral once daily
- Furosemide 20 mg oral once daily
- Losartan 50 mg oral once daily
- Metformin 1000 mg oral twice daily
- Calcium carbonate / Vitamin D3 600 mg / 400 IU oral twice daily
- Acetaminophen 500 mg oral as needed

### Discontinued during this admission:

- Ibuprofen 400 mg oral three times daily as needed - discontinued (nephrology, due to chronic kidney disease)

Follow-up: Primary care in one week; INR recheck per anticoagulation clinic.